



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : حنان حسن ابراهيم الليثي
المحافظة : القاهرة
الحى / المدينة : الجمالية
العنوان : ~
رقم البطاقة : 26810100100801 رقم التليفون : 01150396204
العمر (تاريخ الميلاد) : 10-10-1968 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 06-10-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الأبحاث و الاشعات [7954] قسم العلاج : الاورام
التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.
تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: **خان حسن ابراهيم اللبش**
National ID: 26810100100801



تقرير طبي

السيد

تحية طيبة وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بان نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
--:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Further investigations and imaging

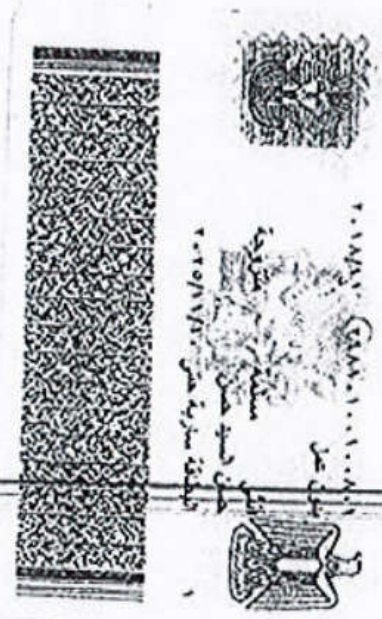
الوقت :

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التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين



Patient Name : الأستاذ / حنان حسن إبراهيم المايشي
Patient No. : 4209-2024
Path No. : 1882/25
Sex : Female
Age : 56 Y
Referring Doc :

Request No. : 13007073
Request Date : 22/05/2025
Verification Date : 19/06/2025
Financial Category : نفقة الدولة
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

Clinical Data : Right breast cancer, Post NACT
Nature of the specimen : Right BCS & Right SLN biopsy

Gross Examination:

- Procedure: Right BCS specimen measured 12x10x2 cm , covering skin ellipse measured 5 x 1 cm .
- Tumor extent : A well defined greyish white TBS measured 1.7x1x1 cm .
- Distance from margins: 1 cm away from lateral margin
- Lymph nodes number and size: A fibrofatty tissue piece measured 5x4x1 cm enclosing 4 SLNs up to 1.5x1 cm .
- Block designation:

1. superior
2. Inferior
3. Lateral
4. Medial
5. Deep
6. TBS
7. SLNS

Crossing by : Dr. Heba Hassan

Microscopic examination:

- Histological type: No residual tumor
- Angioinvasion: No
- Ductal carcinoma in situ: No
- Lymph node involvement: Free SLNs (0/4)
- Resection Margins: Free margins
- Additional pathologic findings: Positive microcalcification

Verified by :
Dr. Heba Hassan

sig Date : 19/06/2025 13:13:24

Page 1 of 2

Printing By : marwa mohamed youss

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Misr El Qadima - Cairo, Egypt

Name: حسان حسن ابراهيم		
MR. : 4459		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمناظرة وجد انه The patient received seven cycles of phesgo, with the last cycle administered in November 2025; treatment was subsequently delayed pending approval from the Ministry of Health. The most recent echocardiogram performed in November 2025 demonstrated a left ventricular ejection fraction of 64 %. For cycle 8, the plan is to continue phesgo together with Arimidex and supportive therapy. A chest X-ray was reported as clear, and other assessments remain pending. The patient reports headache and blurred vision, prompting a request for brain MRI. The patient has completed seven cycles of phesgo. Brain MRI performed on 15 April 2026 was reported as free of significant abnormality. A subsequent chest X-ray on 4 April 2026 was also clear. The patient discontinued phesgo five months ago and is currently receiving Arimidex with supportive treatment for three months.		
تحريرا في ,		
	التاريخ :	19-09-2026
	الوقت :	
	Reham Ahmed Fathy Ibrahim	الطبيب المعالج :
يعتمد ,		

:4209-2024
:1882/25
:Female
:56 Y

Referring Doc :

Request No. : 13007073
Request Date : 22/05/2025
Verification Date : 19/06/2025
Financial Category : دار السلام - نفقة الدولة
Department : Out Patient
Registered by : marwa mohamed

Pathology Report

Diagnosis:

Right BCS & Right SLN biopsy:

No residual viable tumor

• Free LNs (0/4)

• Free margins

• Stage : pT0 , pN0

Dr. Walaa Ibrahim Ghoneim

Dr. Heba Hassan Adly

Verified by :

Dr. Heba Hasan

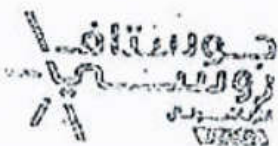
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Page 2 of 2

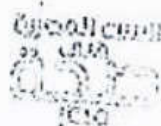
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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo



السلطة الفلسطينية
وزارة الصحة العامة



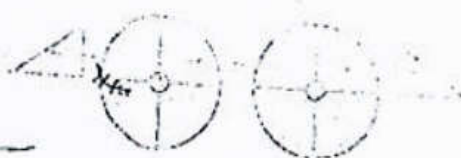
MDT FORM

23/7/2025

Name	Dr. Al-Ramahi	MRN	110.5975.2
Age	57	ID	C.C. / E.C. /
Menopausal status	Post	Phone	110.5975.2
IKS SDO AS		Marital status	Married
BMI		Family history	
CO morbidity	HTN, DM, Bronchial Asth	REASON OF MDT	Post Op

Clinical Data

BX: Pre: IDC GT			
ER:	PR:	HER2:	Ki67:
3/8	3/6	3+	24%
Sonomammography:			
Post: No residual tu			
Mets Workup:			
free n-gins, SLN 0/4.			



yp 6cm

16/6/25
C.C. / E.C. /

Recommendations:

MDT OUTCOME & PLAN

- ① PORT
- ② Cat Phage up to 1+
- ③ AI x 8 cycles

SIGNATURE

Medical Oncology	Surgery	Radiology	Pathology	Nutrition	Psychiatry

Patient Name: Hanan Hassan Ibrahim Al Leithy	Date: 11/12/2024
Exam: C.T CHEST WITH CONTRAST	Age: 56 Year(s)
Medical ID: 4209/2024	Pat. ID: 95292

**POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN AND PELVIS:**

- Reported case of right breast cancer.

I Chest:

- Right breast is partially out of exam field. It shows small parenchymal soft tissue lesion measures about 1.5 cm smaller related similar lesion is noted.
- Small bilateral axillary (level I) lymph nodes, the largest one at the right measures about 1.2 cm and at the left measures about 1 cm.
- Left basal posterior pulmonary nodule measuring 5 mm.
- Prominent subcentimetric mediastinal lymph nodes.
- No pleural or pericardial collection, pleural thickening or masses.
- No cardiac gross enlargement.

II Abdomen & pelvis:

- Enlarged liver showing homogenous parenchymal low attenuation (fatty hepatomegaly). No sizable focal lesions.
- No Intra-hepatic or extra-hepatic biliary radicles dilatation.
- Calcular gall bladder.
- Normal CT appearance of the spleen, pancreas, kidneys, suprarenal glands, aorta and IVC.
- Averagely filled urinary bladder showing no sizable diverticular outpouching.
- Prominent porta-hepatis, porto-caval, para-aortic, aorto-caval and bilateral external iliac lymph nodes, the largest one seen at porta-hepatis region measures about 1.5 x 1 cm.
- No ascites.
- No sizable worrisome osseous lesions.

Opinion:

Reported case of right breast cancer revealed:

- No sizable pulmonary or hepatic metastatic deposits.

Dr. Ali Elsisy, MSc.

Dr. Mohamed Mohsen, M.D.

Dr. Ahmed Fathy

Dr. Hatem Abdel El Abbas

01000010144

01000010144



Patient Name: Hanan Hassan Ibrahim

Patient ID: 255692

Exam Date: 10/12/2024

^{99m}Tc -MDP whole body bone scan

Desired Information:

- Skeletal surveying

Technique:

- Anterior and posterior Delayed Whole body bone scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc -MDP;

Report

Delayed static phase:

- Warm uptake at the sternum of unascertained nature for follow up.
- Degenerative changes are noticed in the dorsal & lumbar vertebrae.
- Arthritic changes at both shoulder and knee joints.
- There is no hot spot that account for osseous deposits could be detected all over the surveyed skeleton.
- Both kidneys are visualized.

Conclusion:

- The surveyed skeleton is free from definite osseous deposits or serious bone lesions.
- Spondylo- degenerative changes at the spine and big joints.

Best regards;
Dr. Huda Fathy M.D.

01000010144



Patient Name: Hanan Hassan Ibrahim Al Leithy	Date : 11/05/2025
Exam : MAMMOGRAPHY BOTH	Age : 56 Year(s)
Medical ID: 4209/2024	Pat. ID: 95292

Combined bilateral breast mammography and complementary ultrasound

Indication:

RIGHT breast pathologically proven malignant mass WITH OVERLYING INSERTED CLIP on NACT for assessment.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR A)
- RIGHT breast shows UOQ irregular RESIDUAL mass lesion, of equal density, with clip seen within.
- Complementary right ultrasound revealed RIGHT breast residual irregular spiculated isoechoic mass lesion, seen at axillary tail with overlying inserted clip, the residual lesion measures about measuring 12 x 10 mm (was 20 x 19 mm, in prior study dated 4 / 11/ 2024), right enlarged axillary LN with preserved fatty hilum
- No left breast solid or cystic masses
- Normal skin thickness, and nipple-areola complex of the right breast.
- Left sided Nonspecific axillary LN,
- Free infra and supra-clavicular regions.

Opinion:

- (Partial) radiological response of the right breast axillary tail malignant mass with overlying inserted clip, as mentioned above (BIRADS 6)
- Benign findings right breast (BIRADS 2)

Dr REHAB EL BADRI

Signature Dr . Rehab El Badry

11/05/2025 2.34 PM

Patient's Name: HANAN HASAN IBRAHIM
Date: 2026-04-15
Referred by: Prof.Dr.
Hospital: Goustave Rousey International Egypt



Bilateral Digital soft tissue mammography with high resolution Breast

sonography revealed:

Clinical summary: Status post right CBS & ALAND, followed by radio therapy, routine follow up study.

Findings:

Right breast:

- Almost entirely fatty breast parenchyma. (ACR a)
- Thickened skin and coarsen trabeculae.
- UOQ, architectural distortion with no speculations, masses, asymmetry, or suspicious microcalcifications.
- Ultrasound demonstrated UOQ area of parenchymal disruption underneath the operative scar with no solid, cystic lesions or fluid collections.
- Thickened skin and coarsen trabeculae.
- Clear right axilla. No pathological infra or supra-clavicular lymph nodes

Left breast:

- Almost entirely fatty breast parenchyma. (ACR a)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
- Ultrasound demonstrated no solid or cystic lesions.
- Normal skin, nipple/areola.
- Non-specific axillary LNs.

Conclusion: -

Right breast (BIRADS 2): Post-Therapeutic sequelae.

Left breast (BIRADS 1): Normal study

Recommendation: -

For annual follow up.

Best Regards,
DR. Amal Hegazi, M.Sc.

Patient's Name:

HANAN HASSAN IBRAHIM

Date:

2026-04-15

Referred by:

Prof.Dr. BRAIN+C

Hospital:

Goustave Roussey International Egypt

MRI EXAMINATION OF THE BRAIN

Technique:

- Axial T1, T2, FLAIR
- Coronal T2
- Sagittal T1
- Axial DWI and ADC
- Postcontrast T1

Findings:

- No cerebral, cerebellar or brain stem areas of altered MR signal.
- Normal size and configuration of the supra and infra tentorial ventricular system.
- No midline shift or deformity.
- No extra axial collections or masses.
- No cerebello pontine angle masses.
- No gross posterior fossa abnormalities.
- Intact cervico medullary junction.
- Bilateral maxillary sinusitis.

OPINION:

➤ Normal MRI of the brain.

Dr.Sherif Wagdy

Dr.Sherif Wagdy

Patient Name: Hanan Hassan Ibraheem		Date : 16/07/2026
Exam : Plain CXR		

Plain X-ray **Chest** (PA view) revealed:

- o *Increased Broncho vascular Markings*
- o *Clear both costo-phrenic angles.*
- o *Normal cardiac size & configuration.*
- o *Normal thoracic cage*

Best Regards,

Dr/ Nermeen Ramadan MD

Patient Name: Hanan Hassan Ibraheem	Date : 16/07/2026
Exam : Ultrasound ABDOMEN& PELVIS	

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver:-

- Enlarged in size (18cm at Rt MCL)homogenous parenchymal echo pattern
- No IHXL seen
- Portal vein is patent and not dilated.

Gall Bladder:-

Disended with Multiple stones smallest measuring 0.8cm

CBD normal

Spleen:-

- Borderline measuring 11 cm, homogenous texture with no focal lesions

Kidneys:-

- Both kidneys of normal size, shape, normal echogenicity and well differentiated with good corticomedullary differentiation.
- No stones no back pressure changes

Pancreatic and pancreatic area: concealed by gaseous distention

Urinary Bladder:-

- Minimally Filled with no stones or masses

Observation

Significant Colonic Gaseous Distention

Hepaticoregally showing Bright color Likely Fatty

Calcified Gallbladder

Name Hanaan hassan Ibraheem

Chest X-Ray Revealed

Lung fields : overall lung fields maintained

Enlarged hilar broncho-vascular markings and Mildly enlarged Cardio- mediastinal silhouette

Diaphragm : hemidiaphragms appear normal in contour

Bones and soft tissue : no acute osseus abnormality identified

Patient's Name: HANAN HASAN IBRAHIM
Date: 2026-04-15
Referred by: Prof.Dr.
Hospital: Goustave Rousey International Egypt



Bilateral Digital soft tissue mammography with high resolution Breast

sonography revealed:

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Findings:

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- Thickened skin and coarsen trabeculae.
- UOQ, architectural distortion with no speculations, masses, asymmetry, or suspicious microcalcifications.
- Ultrasound demonstrated UOQ area of parenchymal disruption underneath the operative scar with no solid, cystic lesions or fluid collections.
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Left breast:

- Almost entirely fatty breast parenchyma. (ACR a)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
- Ultrasound demonstrated no solid or cystic lesions.
- Normal skin, nipple/areola.
- Non-specific axillary LNs.

Conclusion: -

Right breast (BIRADS 2): Post-Therapeutic sequelae.

Left breast (BIRADS 1): Normal study

Recommendation: -

For annual follow up.

Best Regards,
DR. Amal Hegazi, M.Sc.

Patient's Name:

HANAN HASSAN IBRAHIM

Date:

2026-04-15

Referred by:

Prof.Dr. BRAIN+C

Hospital:

Goustave Roussey International Egypt

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Referred by: Prof.Dr.
Hospital: Goustave Rousey International Egypt



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Left breast (BIRADS 1): Normal study

Recommendation: -

For annual follow up.

Best Regards,
DR. Amal Hegazi, M.Sc.

Patient's Name:

HANAN HASSAN IBRAHIM

Date:

2026-04-15

Referred by:

Prof.Dr. BRAIN+C

Hospital:

Goustave Roussey International Egypt

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Date: 2026-04-15
Referred by: Prof.Dr.
Hospital: Goustave Rousey International Egypt



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DR. Amal Hegazi, M.Sc.

Patient's Name:

HANAN HASSAN IBRAHIM

Date:

2026-04-15

Referred by:

Prof.Dr. BRAIN+C

Hospital:

Goustave Roussey International Egypt

MRI EXAMINATION OF THE BRAIN

Technique:

- Axial T1, T2, FLAIR
- Coronal T2
- Sagittal T1
- Axial DWI and ADC
- Postcontrast T1

Findings:

- No cerebral, cerebellar or brain stem areas of altered MR signal.
- Normal size and configuration of the supra and infra tentorial ventricular system.
- No midline shift or deformity.
- No extra axial collections or masses.
- No cerebello pontine angle masses.
- No gross posterior fossa abnormalities.
- Intact cervico medullary junction.
- Bilateral maxillary sinusitis.

OPINION:

➤ Normal MRI of the brain.

Dr.Sherif Wagdy

Dr.Sherif Wagdy

Patient Name: Hanan Hassan Ibraheem		Date : 16/07/2026
Exam : Plain CXR		

Plain X-ray **Chest** (PA view) revealed:

- o *Increased Broncho vascular Markings*
- o *Clear both costo-phrenic angles.*
- o *Normal cardiac size & configuration.*
- o *Normal thoracic cage*

Best Regards,

Dr/ Nermeen Ramadan MD

Patient Name: Hanan Hassan Ibraheem	Date : 16/07/2026
Exam : Ultrasound ABDOMEN& PELVIS	

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver:-

- Enlarged in size (18cm at Rt MCL)homogenous parenchymal echo pattern
- No IHXL seen
- Portal vein is patent and not dilated.

Gall Bladder:-

Disended with Multiple stones smallest measuring 0.8cm

CBD normal

Spleen:-

- Borderline measuring 11 cm, homogenous texture with no focal lesions

Kidneys:-

- Both kidneys of normal size, shape, normal echogenicity and well differentiated with good corticomedullary differentiation.
- No stones no back pressure changes

Pancreatic and pancreatic area: concealed by gaseous distention

Urinary Bladder:-

- Minimally Filled with no stones or masses

Observation

Significant Colonic Gaseous Distention

Hepaticoregally showing Bright color Likely Fatty

Calcified Gallbladder

Name

Hanaan hassan Ibraheem

Chest X-Ray Revealed

Lung fields : overall lung fields maintained

Enlarged hilar broncho-vascular markings and Mildly enlarged Cardio- mediastinal silhouette

Diaphragm : hemidiaphragms appear normal in contour

Bones and soft tissue : no acute osseus abnormality identified

Patient's Name: HANAN HASAN IBRAHIM
Date: 2026-04-15
Referred by: Prof.Dr.
Hospital: Goustave Rousey International Egypt



Bilateral Digital soft tissue mammography with high resolution Breast

sonography revealed:

Clinical summary: Status post right CBS & ALAND, followed by radio therapy, routine follow up study.

Findings:

Right breast:

- Almost entirely fatty breast parenchyma. (ACR a)
- Thickened skin and coarsen trabeculae.
- UOQ, architectural distortion with no speculations, masses, asymmetry, or suspicious microcalcifications.
- Ultrasound demonstrated UOQ area of parenchymal disruption underneath the operative scar with no solid, cystic lesions or fluid collections.
- Thickened skin and coarsen trabeculae.
- Clear right axilla. No pathological infra or supra-clavicular lymph nodes

Left breast:

- Almost entirely fatty breast parenchyma. (ACR a)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
- Ultrasound demonstrated no solid or cystic lesions.
- Normal skin, nipple/areola.
- Non-specific axillary LNs.

Conclusion: -

Right breast (BIRADS 2): Post-Therapeutic sequelae.

Left breast (BIRADS 1): Normal study

Recommendation: -

For annual follow up.

Best Regards,
DR. Amal Hegazi, M.Sc.

Patient's Name:

HANAN HASSAN IBRAHIM

Date:

2026-04-15

Referred by:

Prof.Dr. BRAIN+C

Hospital:

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جمهورية مصر العربية
بطاقة تحقيق الشخصية



حنان
حسن ابراهيم النيش
٣١ درج الطبلوى
الجمالية - القاهرة

١٩٦٨/١٠



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بدون عمل



متزوجة

مسلمة

انثى

حسن السيد حسن

البطاقة سارية حتى ٢٠٣٣/٠٢/١٧

National ID
[DOCTY FIELD BACK]